



POLICY

Inpatient Leave

Scope (Staff):	Clinical Staff
Scope (Area):	CAMHS Acute Inpatient Units

CAHS commits to being a child safe organisation by applying the National Principles for Child Safe Organisations.

Read the full statement here:

[CAHS Child Safe Organisation Commitment Statement](#)

This document should be read in conjunction with this [disclaimer](#)

Aim

To provide guidance to staff around approval, duration and conditions of inpatient leave as well as actions regarding return from inpatient leave.

Risk

- Risk to safety of patients and others.
- Increased readmission rates.
- Prolonged admissions.
- Difficulties managing bed resources.

Background

Inpatient leave is granted for a specified timeframe to assess readiness for community-based treatment and functionality outside of the ward environment as part of a child's discharge plan, or for a specific purpose.

The purpose of inpatient leave may be:

- Therapeutic (to support the child and their family to gradually regain the responsibility of looking after the safety and other needs of the child themselves).
- Assessment (to assess how far the therapeutic gains made during admission can be generalised to the child's home and community).
- Practical (to facilitate the immediate needs of a child which cannot be met while staying within the service; e.g. attending a private medical appointment).

This policy should be read in conjunction with the [Mental Health Act 2014](#) (MHA) (ss. 105-110), the [Clinician's Practice Guide to the Mental Health Act 2014](#) (CPG) and the [PCH Admission and Bed Allocation \(Includes Age of Eligibility and Inpatient Leave\) Policy](#).

Definitions

Inpatient Leave: occurs when a child leaves the ward temporarily with the relevant approval and with the intention that they will return within the timeframe specified by the ward.

Child: an infant, child, adolescent or young person under the age of 16 years.

Family: the person(s) who has parental responsibility for the child. Parental responsibility, in relation to a child, means all the duties, powers, responsibilities and authority which, by law, parents have in relation to children (s. 4 MHA). For a child of Aboriginal or Torres Strait Islander descent, this may be a person from their kinship group who is recognised under customary laws and traditions as their carer (s. 281(2)(b) MHA).

Nominated person: means the person nominated in accordance with Part 16, Division 3 of the MHA. The role of a Nominated person is to assist the child who made the nomination by making sure that the child's rights and interests are taken into account. Nominated persons can be a friend or relative of the child or a child's guardian. It is whoever the child feels should receive information and who can support the patient in relation to their rights, interests and wishes in relation to the functions performed under the MHA. The person nominated must be an adult over the age of 18.

Principles

- CAMHS aims to eliminate the use of restrictive practices in mental health services and act in a person centred and recovery focused way.
- While approved inpatient leave may be extended in the child's absence, granting inpatient leave must not be used as an alternative to discharging the child.
- Delivery of care in an inpatient unit should be kept as brief as possible with the aim to discharge the child back to their family and community at the earliest opportunity.

Procedure

- Consideration must be given to inpatient leave or extension of inpatient leave as part of the clinical assessment and incorporated into the child's Risk Assessment and Management Plan.
- Upon admission, all parents/carers must sign a participation agreement (MR816.18, available through PMG).

- The relevant Shift Coordinator is responsible for confirming that an up-to-date CAMHS [Risk Assessment and Management Plan](#) (RAMP) has been completed and placed in the medical record prior to inpatient leave being approved.
- The child's parent/carer must be fully informed of all ongoing risk issues and actions needed for safety planning, as highlighted in the [CAMHS Risk Assessment and Management Policy](#).
- A copy of the RAMP must be provided to the child and parent/carer. This must outline potential risks of inpatient leave and must be signed by the child, parent/carer and clinician.
- By signing the RAMP, the parent/carer confirms that they understand and accept these risks and agree to follow the safety plan for the duration of the leave.
- Participation must be reviewed and re-assessed on a regular basis, based on changes in clinical risk.

Approval of inpatient leave

- All children require a responsible adult to escort them on inpatient leave. Inpatient leave must be approved by the treating Psychiatrist. Children may not be granted unescorted inpatient leave.
 - Another adult may be identified by the child when there are no family members available to escort the child on inpatient leave. This adult must be approved by the legal guardian.
- The treating Psychiatrist approves inpatient leave if they are satisfied that inpatient leave is likely to benefit the child's recovery from mental illness or mental or physical health.
- Prior to approving inpatient leave, the Psychiatrist, or their delegate must consult with the child's family and/or nominated person (if relevant) in relation to:
 - whether or not to approve inpatient leave; and
 - what period and conditions would be appropriate to specify in the granting of inpatient leave.
- These conditions must be discussed and agreed to by the child and family/nominated person prior to going on inpatient leave. This discussion must include;
 - collaborative development of a plan to monitor the health and wellbeing of the child during the period of inpatient leave; and
 - the actions that parents/legal guardian may have to take (e.g. returning the child to hospital) if the child does not comply with the conditions of inpatient leave.

Approval of inpatient leave for children under an involuntary Inpatient Treatment Order (ITO)

For children under an ITO, the same procedure applies however there are additional requirements as follows:

- The treating Psychiatrist approves inpatient leave by making an order using [Form 7A \(Grant of Leave to an Involuntary Inpatient\)](#) if they are satisfied that inpatient leave is:
 - likely to benefit the child's recovery from mental illness or mental or physical health; and
 - is not inconsistent with the child's need for treatment under an ITO in accordance with s. 105(1)(b) of the MHA.
- The psychiatrist must also consider whether it would be more appropriate to:
 - place the child under a Community Treatment Order (CTO); or
 - make an order revoking the ITO in accordance with ss. 24(6)(c) and 109 of the MHA.
- Form 7A must specify:
 - The date and time it is made;
 - The period of inpatient leave;
 - Any conditions of inpatient leave, including;
 - where the child must reside;
 - treatment the child must receive; or
 - the place(s) the child must attend to receive treatment, whilst on leave in accordance with s. 105(10) of the MHA.
 - The name, qualifications and signature of the psychiatrist in accordance with s. 105(11)(d) of the MHA.
- The psychiatrist must, as soon as practicable, file the Form 7A in the child's medical record.
- A copy must be given to the child, family and/or nominated person (if applicable).

Duration and conditions of inpatient leave

- The duration and conditions of inpatient leave from inpatient services will be determined on the basis of clinical need in consultation with the child, family and/or nominated person.
- The child can only leave the service in the care of a legal guardian or an adult appointed by the legal guardian, unless the child is an adolescent over 16 years

of age, and a voluntary patient who has been assessed and fulfils criteria of a mature minor.

- All inpatient leave must be recorded in the child's medical record.
- Weekend or public holiday leave plans must be clearly documented by the treating team to minimise the need for the on-call team to determine leave plans.
- Overnight leave beds are available for admission, unless the treating Psychiatrist is of the opinion that the bed should be kept vacant for clinical reasons and has clearly identified and documented a rationale as to why the bed cannot be used to admit a new patient.. For further information refer to the [Statewide Mental Health Bed Access, Capacity and Escalation Policy](#).
- Children and their family/nominated person must be informed of the possibility of being moved to an alternative bed or ward on their return from inpatient leave, or of inpatient leave being extended.
- If the child is to be placed on inpatient leave with persons other than the legal guardian, written or verbal permission must be obtained from the legal guardian.
 - Verbal permission must be verified by a second nurse/clinician and verified in the progress notes.
- The child and their family/nominated person should be provided with information for emergency mental health support and the day / time they are expected to return.

Duration and conditions of leave for children under an ITO

For children under an ITO, the same procedure applies however there are additional requirements as follows:

- If the child is an involuntary inpatient, they must abide by the conditions specified in their order granting inpatient leave under Form 7A.
- If an involuntary inpatient is on inpatient leave for a period of more than 21 days, the psychiatrist must consider whether it would be more appropriate to discharge the child or place them under a CTO (s. 108 of the MHA).
- The psychiatrist may vary or extend the period of inpatient leave using [Form 7B \(Extension and/or variation of leave\)](#).
- Form 7B must also be used to vary the conditions of inpatient leave (e.g. the place the child is to reside whilst on leave).
- In completing Form 7B, the psychiatrist must specify the:
 - Date and time of completion;
 - Variation in period/conditions of inpatient leave;

- Reasons for the variation;
- Name, qualifications, and signature of the psychiatrist.
- This order must be filed, and a copy given to the child and family/nominated person as soon as practicable.
- In the case of inpatient overnight leave for involuntary patients, the preferred position is that the child's bed may not be used for new admissions. Exceptions are at the discretion of the treating psychiatrist based on clinical presentation.

Cancellation of Leave for children under an ITO

- If, whilst on leave, a child's risk increases, and it is no longer safe for the child to be on inpatient leave, the psychiatrist may cancel the inpatient leave using [Form 7C \(Cancellation of Grant of Leave\)](#). The child and family/nominated person must be informed as soon as practicable of the cancellation.
- Form 7C must specify the:
 - Date and time the cancellation was made;
 - Reasons for the belief on which the decision to cancel inpatient leave is based; and
 - Name, qualifications, and signature of the psychiatrist.
- Form 7C must be filed in the child's medical record, and a copy given to the child and family/nominated person as soon as practicable.

Return from Leave

- On a child's return from inpatient leave, staff must conduct a mental state examination and risk assessment. A search of belongings must be completed where indicated.
 - See [CAMHS Risk Assessment and Management Policy](#), [Ward 5A Progressive Risk Monitoring Work Instruction](#) and the [Searching Patients and their Belongings Policy](#).
- If a child does not return from inpatient leave at the specified time, staff must follow the [PCH Absconding / Missing Patients Policy](#).
- Every effort should be made to return the patient to the authorised hospital before an [Apprehension and Return Order \(Form 7D\)](#) is made. For instance, where a patient has wandered away from an authorised hospital while on ground leave, or is unaware of when inpatient leave expired, or is unaware the inpatient leave was cancelled, making an Apprehension and Return Order should never be an automatic response. Refer to section 4.10 of the CPG for further guidance.
- On a child's return from inpatient leave, staff must update the child's medical record and PSOLIS.

References and related external legislation, policies, and guidelines
Department of Health Statewide Mental Health Bed Access, Capacity and Escalation Policy
Mental Health Act 2014
Clinician's Practice Guide to the Mental Health Act 2014
Related CAHS internal policies, procedures and guidelines
PCH Admission and Bed Allocation (Includes Age of Eligibility and Inpatient Leave) Policy
CAMHS Risk Assessment and Management Policy
Ward 5A Progressive Risk Monitoring Work Instruction
Searching Patients and their Belongings Policy
PCH Absconding / Missing Patients Policy.
Useful resources (including related forms)
SSCD Risk Assessment and Management Plan (RAMP)
Form 7A (Grant of Leave to an Involuntary Inpatient)
Form 7B (Extension and/or variation of leave).
Form 7C (Cancellation of Grant of Leave).
Form 7D (Apprehension and Return Order)

This document can be made available in alternative formats on request.

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